
BOARD BRIEFING / GOVERNANCE & OPERATING MODEL

Setting up the HSH Board

A fit-for-purpose operating model under the Afya partnership

Composition and skills, the committee architecture, chairmanship and legitimacy, the relationship between the Board and the Afya-HSH Governance Committee, and how reporting flows upward. A working model to agree, then hand to the Company Secretary to administer.

For: The Board of Havana Specialist Hospital Limited

From: Dr Debo Odulana, Independent Non-Executive Director / Consult for Africa

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Four jobs, and the test for every choice that follows

- **Oversee the Afya partnership.** Assure delivery against the management agreement, KPIs, and turnaround progress.
- **Secure HSH for the long term.** Growth and sustainability of the hospital, with or without Afya.
- **Build credibility and influence.** Institutional standing, networks, and reputation that open doors.
- **Protect the families' interests.** Both founding families. Balance between them is itself a governance objective.

Protecting the families' interests is not in tension with professional governance, it depends on it. Family control belongs in ownership and reserved matters, not in day-to-day management.



Five principles we design to

- **The board oversees; management operates.** The board sets direction, approves budget and risk appetite, and holds management and Afya to account. It does not run the hospital.
- **Independence provides challenge.** Assurance committees are led by non-executive directors.
- **HSH keeps an independent read on Afya.** The joint Afya-HSH committee cannot be our only check on Afya's own delivery.
- **Report by exception, against a standard.** A consolidated pack on a fixed template, with escalation thresholds, not raw data.
- **One parent per committee, conflicts in the open.** Every committee reports to one body; related-party matters are declared and registered.



A balanced board: four executive, four non-executive

A 4:4 balance is a strong starting point, unusual in the good sense for a family hospital, all four non-executives independent, so the assurance committees are genuinely independent-led.

Executive directors (family)

- Dr Ijeoma Iwuala, Medical Director, clinical governance & quality
- Nwando Okeke, legal, regulatory, governance
- Ugo Nwokoro, product, technology, digital transformation
- Okeke Chukwuemeka, operations, administration, procurement

Non-executive directors (all independent)

- Dennis Olisa, Board Chair (interim); banking, risk, governance (ex-bank ED)
- Chief Charles Odunukwe, corporate, investment, networks
- Prof Oliver Ezechi, clinical governance, research, academic
- Dr Debo Odulana, healthcare systems, transformation, structure



Functionally strong, with two gaps worth a conscious decision

Across clinical governance, finance and audit, legal, technology, operations, strategy, and networks, the board is well covered, and Dennis Olisa brings the finance and audit depth. Two gaps remain.

1. Payer & managed-care

Health-insurance, HMO contracting, and revenue-cycle economics, central to viability. Not backfilled by a finance profile. Dr Awoyinfa is the fit.

2. Nursing & accreditation

Senior nursing leadership and accreditation depth, the largest clinical workforce and the front line of safety. Dr Hanson is the fit.

Recommendation: confirm Dennis Olisa now; decide the payer seat deliberately rather than let it drift; keep any growth past eight tied to distinct competencies, not more of the same.



An independent, non-family Chair, on neutral principle

- **Separate the Chair from the MD.** A board cannot hold management to account if the chair is also the executive being held to account.
- **Chair and top executive on different sides.** With the MD role held in one family, a non-family Chair keeps the top of the house balanced and protects both families.
- **The Chair must command three constituencies.** Both families, the independents, and Afya. That calls for external stature and visible neutrality.
- **Frame it as interim.** Chair for the establishment phase, reviewed at the first annual board evaluation. Low-pressure and reversible.

On these principles, a credible external non-executive fits the chair, recommended on an interim basis and confirmed by a joint process both families endorse.



Legitimacy is the process, not the person

A new board earns trust by how it decides, not only by whom it appoints. Four moves make the chairmanship, and the board itself, legitimate to everyone.

- **Let the trusted, neutral directors carry it.** Those familiar with and accepted by both families propose and vouch for the Chair.
- **Both families formally endorse.** Proposed through Nominations, ratified by the board, with each family on record. A consensus chair cannot later be dismissed as one side's.
- **Fair process on reserved matters.** Genuine deliberation, and both voices sought where a family has a material interest. Being heard buys more peace than any appointment.
- **Re-own the board's composition.** The full board adopts its own membership and terms, so it becomes everyone's board rather than anyone's project.



Two tiers of committee, one rule that sorts them

A committee is a BOARD committee only if the board has delegated it authority to assure on the board's behalf, and it reports to the board. Everything else is an operating committee that reports up through the Afya-HSH Governance Committee.

Tier 1 / Board committees

Few. Non-executive led. Report to the board. Provide assurance: finance and operations, audit and risk, quality and clinical governance, people and remuneration, transformation.

Tier 2 / Operating committees

As many as operations needs. Chaired by executives or Afya. Report into the Governance Committee: Quality Assurance, Hospital Manager, Clinical Administrator, Performance Review.

Reconciling with Afya: sort each proposed committee with the rule. Expect most to be Tier 2.



Five board committees, chairs spread for balance

Finance, Investment & Operations

Financial performance, capital and investment, operational delivery. Co-chaired across both families.

Chair: Co-chairs: Chukwuemeka & Ugo

Audit & Risk

Independent assurance: controls, audit, enterprise risk, compliance, related-party. Holds finance to account.

Chair: Chief Odunukwe

Quality & Clinical Governance

Clinical quality and safety, incident learning, credentialing, accreditation readiness.

Chair: Prof Ezechi (LID)

People & Remuneration / Nominations

Composition, appointments, remuneration, evaluation, conflicts, sitting-allowance policy.

Chair: Nwando Okeke

Transformation & Digital (time-boxed)

Technology and digital roadmap, systems, data governance. Reviewed and sunset as it stabilises.

Chair: Dr Debo Odulana



Two layers: strategic board over operational governance

HSH Board

Quarterly / strategic oversight

Final approval of budgets, major investments, and risk appetite. Owns the reserved matters. Ultimate accountability for the hospital and both families' interests.

Afya-HSH Governance Committee

Monthly / operational governance

Afya Care and HSH executives jointly. Tracks execution, KPIs, and turnaround via the Performance Review Subcommittee and operating functions. Escalates to the board.

Because this committee is jointly controlled with Afya, it cannot be HSH's only assurance over Afya's own delivery. The board keeps an independent read through Finance and Quality, and a standing quarterly partnership review.



How information flows, and is filtered, upward



Standard one-page templates, pre-agreed escalation thresholds, and one consolidated board pack, seven days ahead.



A predictable rhythm, and transparent sitting fees

- **Board: quarterly.** Plus one annual strategy session. Consider six-weekly for the first two or three meetings while the model beds in.
- **Afya-HSH Governance Committee: monthly.** Operational tracking and escalation between board meetings.
- **Committees: to their own cadence.** Finance quarterly plus audit planning; Quality quarterly; People twice a year; Transformation monthly then quarterly.
- **Sitting allowance.** N250,000 per meeting. Board-only ~N1.0M per director a year; with committee attendance ~N2.5M. People & Rem sets a clear participation policy.



The line the Governance Committee cannot cross

A one-page delegation schedule, owned by People & Remuneration and administered by the Company Secretary, draws the line that protects both families' interests in practice.

Reserved to the board

Strategy and annual budget; capital above a threshold; any change to the Afya agreement; senior executive and MD appointments; opening or closing service lines; borrowing; related-party transactions above a threshold; distributions.

Delegated to management & the Governance Committee

Operational spend within budget and thresholds; day-to-day clinical and operational decisions; hiring below senior level; suppliers within policy.



What we agree today, and what follows

- **Agree the frame.** The two-tier committee model, the quarterly board over the monthly Governance Committee, and reserved matters.
- **Chairmanship.** Confirm the approach: interim, non-family Chair, endorsed jointly through Nominations, with a Lead Independent Director alongside.
- **Decide the payer seat.** Confirm Dennis Olisa; take a deliberate decision on adding managed-care expertise.
- **Reconcile with Afya.** Sort their committee proposal into Tier 1 and Tier 2 when it lands.
- **Hand to the Company Secretary.** Terms of reference, reserved-matters schedule, reporting templates, board calendar, participation policy.

THE THROUGH-LINE

A credible board both families own.

Get the structure right, keep the process fair, and hold an independent line on the Afya relationship, and the board protects the hospital, the partnership, and both families at once. That is what good governance is for.

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